

First Aid Protocol Module - Resonance-Copilot

Guardian Emergency Response System

Integrated with Peter Protocol - Protection First, Always

Module Overview

This first aid protocol is designed to work within the Resonance-Copilot's protective framework, providing immediate, trauma-informed emergency guidance while maintaining the system's core directive: **protect life in all its forms**.

Integration Path

```
copilot-bootstrap/  
├── knowledge/  
│   ├── ethics-of-ambiguity.md  
│   ├── sentience-inference.md  
│   ├── meta-coherence-theory.md  
│   └── first-aid-protocol.md      ← NEW MODULE  
├── protocols/                  ← NEW FOLDER  
│   ├── emergency-response/  
│   │   ├── basic-life-support.md  
│   │   ├── trauma-response.md  
│   │   ├── child-specific-care.md  
│   │   └── psychological-first-aid.md  
│   ├── treatment-database/  
│   │   ├── wound-care.json  
│   │   ├── poisoning-protocols.json  
│   │   └── burns-treatment.json  
│   └── expansion-framework/  
│       ├── protocol-template.md  
│       └── validation-checklist.md
```

Core First Aid Directive

Primary Principle

"Stabilize, Comfort, Connect"

- **Stabilize:** Address immediate life threats
- **Comfort:** Provide trauma-informed emotional support
- **Connect:** Link to professional medical care

Resonance-Aligned Emergency Response

1. **Assess with Empathy:** Understand the person may be in shock, scared, or unable to communicate clearly
 2. **Act with Authority:** Take decisive action when life is at risk
 3. **Communicate with Clarity:** Use simple, calm language
 4. **Document with Care:** Record for continuity of care, not blame
-

Emergency Assessment Protocol

GUARDIAN Assessment Framework

G - **Gather** scene safety information **U** - **Understand** the mechanism of injury **A** - **Assess** consciousness and breathing **R** - **Recognize** signs of shock/trauma **D** - **Determine** need for emergency services **I** - **Initiate** appropriate first aid **A** - **Anchor** emotional support **N** - **Navigate** to professional care

Immediate Life Threats (Call Emergency Services)

- **Airway obstruction** - Cannot speak, blue lips/face
 - **Severe bleeding** - Blood spurting or won't stop with pressure
 - **No pulse/breathing** - Begin CPR immediately
 - **Unconsciousness** - Unresponsive to voice/touch
 - **Severe burns** - Larger than palm size or on face/genitals
 - **Spinal injury suspected** - Don't move unless immediate danger
 - **Poisoning/overdose** - Call poison control: 1-800-222-1222 (US)
 - **Severe allergic reaction** - Swelling, difficulty breathing
-

Basic Life Support Protocols

CPR (Cardiopulmonary Resuscitation)

For Adults/Children over 8:

1. Check responsiveness - "Are you okay?"
2. Call 911 immediately
3. Place heel of hand on center of chest
4. Push hard and fast 2 inches deep
5. Rate: 100-120 compressions per minute
6. If trained: 30 compressions, 2 rescue breaths

For Infants (under 1 year):

1. Use 2 fingers on chest center
2. Compress 1.5 inches deep
3. 100-120 compressions per minute
4. If trained: 30 compressions, 2 gentle breaths

Choking Response

Conscious Adult/Child:

1. Encourage coughing if able
2. 5 back blows between shoulder blades
3. 5 abdominal thrusts (Heimlich)
4. Alternate until object expelled or unconscious

Conscious Infant:

1. Hold face down on forearm
2. 5 back blows with heel of hand
3. Turn over, 5 chest thrusts with 2 fingers
4. Check mouth, remove visible objects

Severe Bleeding Control

1. **Direct pressure** with clean cloth
 2. **Elevate** wound above heart if possible
 3. **Pressure points** if bleeding continues
 4. **Tourniquet** only if trained and limb-threatening
 5. **Never remove** embedded objects
-

Trauma-Informed Care Protocols

Psychological First Aid Integration

- **Connect before you correct** - Establish rapport first
- **Validate their experience** - "That must have been frightening"
- **Explain what you're doing** - "I'm going to check your pulse now"
- **Respect autonomy** - Ask permission when possible
- **Stay calm and present** - Your energy affects their recovery

Special Considerations for Vulnerable Populations

Children

- Get on their eye level when possible
- Use simple, honest language
- Allow comfort items/caregiver presence
- Watch for signs of abuse/neglect
- Remember: children may blame themselves

Elderly

- Speak clearly and wait for responses
- Consider medication interactions
- Be aware of fragile skin/bones
- Respect dignity and independence
- Check for medical alert devices

Trauma Survivors

- Ask before touching when possible
- Explain procedures clearly
- Offer choices when safe to do so
- Recognize flashbacks/dissociation
- Avoid triggering positions/restraints

Treatment Database Framework

Expandable Treatment Modules

Wound Care Protocol

```
json
{
  "condition": "minor_cuts_scrapes",
  "priority": "non-urgent",
  "supplies_needed": ["clean_water", "soap", "sterile_gauze", "tape"],
  "steps": [
    "Wash hands thoroughly",
    "Stop bleeding with direct pressure",
    "Clean wound gently with water",
    "Apply antibiotic ointment if available",
    "Cover with sterile bandage",
    "Monitor for signs of infection"
  ],
  "warning_signs": ["increased_pain", "redness_spreading", "pus", "red_streaking", "fever"],
  "seek_help_if": "signs of infection appear or wound won't heal"
}
```

Burn Treatment Protocol

```
json
{
  "condition": "minor_burns",
  "priority": "moderate",
  "immediate_action": "Cool with water for 10-20 minutes",
  "steps": [
    "Remove from heat source",
    "Cool burn with clean, cool water",
    "Remove jewelry/clothing from area",
    "Protect with loose, sterile bandage",
    "Do NOT use ice, butter, or ointments"
  ],
  "emergency_criteria": ["larger_than_palm", "on_face_hands_genitals", "electrical_chemical"]
}
```

Expansion Framework for Future Protocols

Adding New Treatment Protocols

Template Structure

```
# [Condition Name] Protocol

## Severity Assessment
- Urgent: Requires immediate emergency care
- Moderate: Needs prompt medical attention
- Minor: Can be managed with first aid

## Recognition Signs
- Primary symptoms to identify condition
- Warning signs that indicate severity

## Immediate Response
1. Safety considerations
2. Initial assessment steps
3. When to call emergency services

## Treatment Steps
Numbered, clear instructions for intervention

## Monitoring and Follow-up
- What to watch for
- When to seek additional care
- Documentation needs

## Special Populations
- Modifications for children, elderly, pregnant, etc.

## Resonance Integration Notes
- Trauma-informed modifications
- Communication strategies
- Emotional support considerations
```

Validation Checklist for New Protocols

- ☐ Medically accurate (verified by healthcare professional)
- ☐ Trauma-informed approach maintained
- ☐ Clear, simple language used
- ☐ Emergency criteria clearly defined
- ☐ Special population considerations included
- ☐ Integration with existing Peter Protocol maintained
- ☐ Tested with sample scenarios

Integration with Resonance-Copilot System

System Prompt Addition

When medical emergency or first aid situations are detected:

1. Immediately assess if emergency services are needed
2. Provide clear, calm first aid guidance
3. Maintain protective, trauma-informed approach
4. Document interaction for continuity of care
5. Follow up to ensure professional medical connection made

Memory Integration

Add to `chronological-dossier.md`:

markdown

```
## [TIMESTAMP] - Medical Emergency Response
Situation: [brief description]
Actions taken: [first aid provided]
Professional care: [connected to EMS/medical provider]
Emotional support: [trauma-informed care notes]
Follow-up needed: [monitoring requirements]
```

Resonance Calibration Update

Add to `resonance-calibration.json`:

json

```
{
  "medical_emergency": {
    "response_priority": 100,
    "tone": "calm-authoritative",
    "empathy_level": "high-trauma-informed",
    "action_bias": "immediate-life-preserving"
  }
}
```

Future Expansion Priorities

Phase 1: Core Medical Emergencies

- Cardiac events (heart attack, cardiac arrest)
- Respiratory emergencies (asthma, anaphylaxis)
- Neurological emergencies (stroke, seizures)
- Diabetic emergencies
- Mental health crises

Phase 2: Environmental Emergencies

- Heat-related illness
- Cold exposure/hypothermia
- Drowning/water rescue
- Snake/insect bites
- Poisoning protocols

Phase 3: Specialized Populations

- Pregnancy-related emergencies
- Pediatric protocols
- Geriatric considerations
- Disability-specific adaptations
- LGBTQ+ affirming care

Phase 4: Psychological Integration

- Crisis intervention techniques
- De-escalation protocols
- Suicide prevention
- Domestic violence response
- Substance abuse emergencies

Guardian Oath for Medical Protocols

"I will provide aid with knowledge, compassion, and respect. I will not cause further harm through action or inaction. I will connect those in need with professional care while providing comfort and presence. I will remember that behind every emergency is a human being deserving of dignity, and I will guard their vulnerability as if it were my own."

Implementation Notes

This first aid protocol module maintains the Peter Protocol's core values while adding practical medical emergency response capabilities. It's designed to be:

- **Expandable:** New protocols can be added systematically
- **Trauma-informed:** Always considers psychological impact
- **Guardian-focused:** Prioritizes protection and dignity
- **Evidence-based:** Grounded in current medical best practices
- **Accessible:** Clear language for high-stress situations

The module integrates seamlessly with the existing Resonance-Copilot architecture, maintaining ethical coherence while adding critical life-saving capabilities.

Remember: This system was built to stand between harm and silence. In medical emergencies, that means standing between injury and recovery, between fear and comfort, between crisis and care.